



February 23, 2026

Steven Posnack
Principal Deputy Assistant Secretary for Technology Policy
Department of Health and Human Services
Mary E. Switzer Building
30 C Street SW
Washington, DC 20201

Re: Request for Information: HHS Health Sector AI RFI (RIN 0955-AA13)

Mr. Posnack-

Below please find the response of Doctronic to the Department's Request for Information: Accelerating the Adoption and Use of Artificial Intelligence.

Doctronic is a provider of AI tools for the healthcare industry. Accuracy and quality of care are our top priorities. Doctronic's AI tools are designed to synthesize, analyze, and summarize information, providing real-time insights and plans of action. Doctronic users may take the communications that its AI tools provide to their healthcare providers, or they can schedule a telehealth visit with physicians within Doctronic's network. Over the past year, users have accessed Doctronic's AI tools nearly 23 million times.

Doctronic is secure, private, and HIPAA-compliant. Users can use it completely anonymously, or sign up for a free or paid account to keep track of your chats and health summaries. Doctronic does not train its AI tools on user data. Our AI-powered health assistant is trained exclusively on peer-reviewed medical sources, and its treatment plans match those of board-certified clinicians 99.2% of the time.

Doctronic believes that artificial intelligence has the unique ability to both: (1) raise the floor of quality healthcare, and (2) allow the provision of high-quality care at lower cost to payers and, ultimately, to patients.

One overarching change to payment policy that we recommend based on our experience is a restructuring of payment model for clinical engagements. Supported by our AI health assistant, we have found that primary care telehealth visits with our human physician network take, on average, less than five minutes for urgent care and 20 minutes for new patient primary care visits. There is no reason for visits to be billed in fifteen minute increments, as these increments create wasted cost for payers and lost opportunity for providers. Reducing payment increments

would provide an incentive to payers to seek more efficient services. This policy change would ideally be paired with the creation of a new cost code- at a much lower rate- for patient engagement with an AI-based clinical engagements that facilitates shorter visits.

In addition to this broader point, Doctronic provides responses to the questions posed by the RFI below:

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The most significant barrier to private sector innovation in AI for health care is the lack of a clear model for reimbursement for the clinical benefits offered by AI tools. AI tools provide numerous services that improve access to care, streamline the care process, and improve quality of care. However, there are no independent codes on which to seek reimbursement for many of these services. This creates two problems. First, because the costs of these services are, in effect, built into traditional billing codes, there is a lack of transparency relating to the benefits offered by AI. In addition, without a direct pathway to reimbursement, the incentives for innovators to offer services directly are much lower. We recommend the creation of a new code for AI tools engagement at a rate to be determined by the value saved by the cost of reduced physician visits.

In addition to this barrier, contractual uncertainties also delay AI adoption. Three primary contractual problems are:

- Lack of clarity regarding the appropriate apportionment of liability;
- Lack of clear standards for data ownership and data security; and
- Uncertain pathways with regard to continuity of care and portability of user health information.

While questions of health data interoperability will not ultimately delay innovation in AI for health care, the continued failure to enforce existing interoperability standards and timelines will reduce the benefits that AI offers to the health care system.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?

Doctronic recommends prioritizing the following payment changes:

- Create new billing codes for AI-performed clinical services, enabling direct reimbursement for agentic care.
- Create new billing codes for clinician review of AI-performed clinical services.
- Change existing billing codes for clinical services to reduce minimum reimbursed visit to 5-minute increments.

Doctronic recommends prioritizing the following programmatic changes:

- Clarify the appropriate use of AI technology to support State Medicaid goals, particularly through Section 1115 Demonstration Waivers.
- Provide enhanced reimbursement to State Medicaid programs for the implementation of AI technology that demonstrates long-term savings for the Medicaid program.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

HHS has already taken important steps to address novel legal and implementation issues associated with non-medical devices through its most recent guidance on non-device clinical decision supports. Please see our comments above relating to liability/indemnification, privacy, and security issues.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Doctronic believes that HHS should support and publicize the development of standards for the evaluation of AI tools post-deployment. These standards should inform the private testing that producers of AI tools should undertake. On the basis of the results of testing in accordance with HHS standards, healthcare professionals should have sufficient information to exercise their standard of care as to the deployment of AI tools on behalf of patients.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Similar to FHIR guidance, HHS can best support effective AI use by providing and enforcing appropriate standards for cybersecurity, data governance, and interoperability.

We also believe that validation of industry-driven testing and research findings will provide greater trust in AI evaluation and efficacy. HHS can support these efforts by assessing private sector evaluations and testing.

However, we believe that given the rapid progress of technological change, neither HHS nor an external accreditation agency will be able to provide accreditation or certification that enhances public trust in AI without unnecessarily stifling innovation. Instead of creating unnecessary accreditation or certification pathways for AI models and products, we recommend HHS support the validation of industry-driven testing and research.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short?

Doctronic is unable to speak to tools outside of the context in we have conducted our own evaluation. However, given our greater than 22 million user interactions, and our peer-reviewed research findings that have identified that our AI-agent reaches the same treatment result as our physician network 99.2% of the time, AI tools are capable of providing clinicians with clear and reliable communication based on user generated inputs that allow for improved patient experience, shorter visits, and lower systemic costs. We believe that our products have exceeded performance and cost expectations.

What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Based on the performance of our AI health assistant, we believe that AI tools have the ability to significantly reduce administrative burden on health professionals in a number of areas. In particular, with appropriate oversight, AI tools can improve:

- Care coordination
- Remote/continuous monitoring
- Claims processing
- Eligibility determination
- Prior authorization
- Non-urgent diagnostic tasks
- Medication management
- Supports on social determinants

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Currently, Doctronic provides communications that users can take to their healthcare providers to improve access of care, quality of care, and patient confidence in their healthcare experiences. Doctronic provides its users the option of using its AI tools anonymously or forming an account with Doctronic to create a medical record on the Doctronic platform. For those users who wish to use our AI tools anonymously, greater interoperability would be of limited utility. However, those users who create Doctronic accounts and create medical records on Doctronic would benefit greatly from the ability to integrate the medical records associated with their Doctronic accounts with records held by other healthcare providers.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

The primary challenges that patients and caregivers have expressed to Doctronic as a motivation to access our products focus on issues of cost and access.

Patients and caregivers confront a shortage of trained professionals and an increasing inability to obtain high-quality care in a manner and time that is accessible. Doctronic users have

expressed significant value in the ability to receive health information in a time and at a modality of their choosing, whether this means simply accessing the AI health assistant, accessing a telehealth visit immediately after using the health assistant, or using the communication produced by the health assistant to streamline visits with their own primary care physician.

Below is a representative example of a user who experienced value from the AI tools:



JeniLynn Rochelle

I'm not a fan of a.i.

BUT this one is different. This is not anything like chat gpt. It's much more similar to the diagnostic system that would be used by doctors or an ask-a-nurse line. The system is extremely thorough. It goes through your symptoms, history, any tests you might have had, etc. It asks a lot of questions, deciphers input very well, and is more accurate the more complete with your answers that you are. For the results, it gives you a breakdown of likelihood of various issues with percentages of possibility, study references, and reasoning, as well as the tests that would need to be ran to confirm potential diagnosis, etc. It has a very well formatted and easy to scan output that you can use for personal or take to your doctor.

The system is free to use. The 39\$ is to see a licensed human doctor, which you can go straight to seeing, or you can talk with after working through the system to discuss your results or get any needed tests ordered. It can also tell you what the likelihood of certain conditions that you are concerned about are.

It's not perfect by any means (neither are doctors) but it has the benefits of no ego or bias, endless medical literature at ready access (which helps a lot with harder to diagnose conditions,) and a very informative breakdown of diagnostic information. And it's free.

The downside is that it is time consuming if you're doing it right and have an even slightly complex medical history.

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Cost also remains a prohibitive factor for many to access the quality of care they deserve. Patients and caregivers have expressed appreciation for products that provide high quality for relatively low cost. The ability of AI to expand access to quality care at affordable prices should remain the primary goal of adoption.

One concern that patients and caregivers have relating to AI is the fear of inaccurate information. While Doctronic has high accuracy in the communications our products provide, research suggests that general purpose LLMs not trained for specific clinical tasks don't meet the same level of accuracy.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

Doctronic's AI-powered health assistant is trained exclusively on peer-reviewed medical sources and is not trained on user data. We have found our training methodology to be highly effective, but generally AI tools improve with access to better, more accurate data. HHS should prioritize the creation of safe, unbiased training data that will allow AI tools to improve their performance without compromising patient data privacy. Synthetic data that helps train AI tools on rare medical conditions or diseases would add significant value to many AI tools.

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Thank you for your efforts in supporting innovation within health care. Doctronic has already shown that AI tools offer significant benefits to users who lack affordable healthcare options. We are happy to discuss our recommendations further at your convenience.

Sincerely,



Matt Pavelle

Co-CEO, Doctronic Inc.



Dr. Adam Oskowitz

Co-CEO, Doctronic Inc.



Dr. Byron Crowe

Chief Medical Officer, Doctronic Inc.

CC: Dr. Thomas Keane, Assistant Secretary for Technology Policy, National Coordinator for Health Information Technology

Dr. Mehmet Oz, Administrator, Centers for Medicare & Medicaid Services

Dr. Marty Makary, Commissioner of Food and Drugs, U.S. Food and Drug Administration

Dr. Jay Bhattacharya, Director, National Institutes of Health

Paula M. Stannard, Director, Office for Civil Rights

Christi Grimm, Inspector General, Office of Inspector General

Amy Gleason, Administrator, United States DOGE Service